THE WHOLE TREATMENT JOURNEY

Weekly paclitaxel + trastuzumab → trastuzumab continuation: the whole pathway

Adjuvant · HER2-positive early breast cancer

Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	After surgery	Combination · 12 weeks	Trastuzumab continuation	Completion	Follow-up
Cancer medicines NCCP 00815	Confirm HER2 and indication	Paclitaxel weekly + trastuzumab every 3 weeks	Trastuzumab every 3 weeks	Complete one year total if appropriate	Endocrine treatment if HR-positive
Surgery EARLY	Surgery usually already completed	Wound / arm recovery	No routine further operation from this regimen	MDT review if needed	Report breast / chest-wall changes
Radiotherapy EARLY	Agree indication and fields	Usually after paclitaxel	Can overlap trastuzumab if agreed	Review skin, fatigue and breathing	Late effects assessed if symptoms
Blood tests NCCP 00815	FBC, kidney/liver tests	Before each weekly chemotherapy dose	According to clinical status / ongoing treatment	Review unresolved toxicity	Only as clinically indicated
Heart checks NCCP 00815 · ESC	Baseline echo / LVEF	Every 12 weeks during HER2 treatment	Continue the same surveillance clock	Completion assessment / risk review	Follow-up by cardiac risk
Cancer imaging EARLY · BONE	Review operative pathology	No routine CT series if well	Routine surveillance plan	Confirm first mammogram timing	Annual mammography; symptoms prompt assessment

What each check is for

1

Blood tests and treatment review

FBC, kidney/liver tests → Before each weekly chemotherapy dose → According to clinical status / ongoing treatment → Review unresolved toxicity → Only as clinically indicated

Results may change whether a dose is given or held.

Ask / act: Confirm the booking and who reviews the result before treatment.

Sources: NCCP 00815

2

Heart tests

Baseline echo / LVEF → Every 12 weeks during HER2 treatment → Continue the same surveillance clock → Completion assessment / risk review → Follow-up by cardiac risk

ECG measures rhythm; echo measures pumping function. They are different checks.

Ask / act: Keep the planned appointments and report symptoms between them.

Sources: NCCP 00815 · ESC

3

Imaging and markers

Review operative pathology → No routine CT series if well → Routine surveillance plan → Confirm first mammogram timing → Annual mammography; symptoms prompt assessment

The purpose and frequency depend on early versus advanced disease.

Ask / act: Ask which test is next and what decision its result will inform.

Sources: EARLY · BONE

4

Bone health with endocrine treatment

At endocrine / ovarian-suppression planning; repeat by risk

Low oestrogen can affect bone strength.

Ask / act: Ask whether DEXA, fracture-risk assessment or bone protection is appropriate.

Sources: BONE · NOGG

5

Who arranges and reviews each check?

At initiation and every handover

A recommended test can be missed if responsibility is unclear.

Ask / act: Ask the team to document the booking owner, result reviewer, due window and escalation route.

Sources: NCCP 00815

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-17 · Draft — independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.83.8